MEDICONNECT HOSPITAL MANAGEMENT SYSTEM

Project Closure Report

Version 1.0 | Prepared 10 November 2026

SELF-DIRECTED PROJECT SIMULATION — NOT A REAL HOSPITAL IMPLEMENTATION

1. Purpose and closure status

This report records the proposed close-out position for the MediConnect learning project. MediConnect is a simulated hospital management system project developed as a practical exercise in project coordination and business analysis. No real hospital implementation, production deployment, user acceptance approval or formal sponsor sign-off has taken place.

Closure status: Documentation and planning exercises are substantially prepared. Actual project delivery cannot be confirmed from the available evidence. This report is therefore a portfolio closure exercise and a proposed closure record, not a claim that a live project has been successfully completed.

2. Project summary

The proposed project is a web-based Hospital Management System intended to address assumed challenges associated with paper-based patient records, manual appointment handling, billing processes and limited information sharing between departments. These problems were used as case-study assumptions and have not been validated through interviews with a real hospital.

Item	Project information
Project name	MediConnect – Hospital Management System
Project type	Self-directed project simulation
Proposed delivery approach	Minimum Viable Product (MVP), followed by possible future phases
Proposed initial delivery target	Three months, as stated in the project charter
Detailed schedule	The Gantt workbook currently represents approximately 21 calendar weeks; this needs reconciliation with the charter target.
Prepared project role	Project Coordinator / Business Analyst practice role
Sponsor / approval	Not assigned; no real sponsor approval recorded

3. Objectives and intended outcomes

- Improve patient registration and appointment scheduling.
- Make patient records easier for authorised clinical staff to retrieve and update.
- Reduce manual billing and payment-record errors.
- Improve information sharing between departments, subject to role-based access.
- Provide management with operational summaries and reports.
- Support appropriate protection, backup and auditability of patient information.

4. Scope and deliverables review

Planned deliverable	Evidence prepared	Closure position

Project initiation and charter	Project Charter	Draft prepared; sponsor approval fields remain blank.
Business requirements	Business Requirements Document	Initial working draft prepared from simulated stakeholder responses; requires real stakeholder validation.
Work breakdown structure	WBS	Planning draft prepared; estimates and dependencies require review.
Schedule	Excel Gantt Chart	Illustrative schedule prepared; differs from the charter's three-month target.
Progress monitoring	Weekly Status Report (Word and Excel)	Illustrative reporting templates prepared; sample progress values are not actual measurements.
Risk and issue management	Risk Register and Issue Log	Practice register prepared; owners and ratings are proposed, not formally accepted.
User acceptance testing	UAT Plan and Sign-off Template	Test scenarios and acceptance template prepared; no actual test results or sign-off recorded.
Deployment and handover	Deployment, Training and Handover Plan	Proposed readiness, training and handover approach prepared; no deployment or handover evidenced.

5. Acceptance criteria and evidence position

The project charter identifies the following intended acceptance conditions. The current portfolio files define how these could be checked, but do not prove that they have been met.

- Agreed MVP features are developed and operational.
- Patient registration, appointment management, authorised medical-record access, billing and management reporting work as required.
- Role-based access and required security checks have been tested.
- Critical defects identified during testing have been resolved.
- Hospital representatives complete UAT and approve the system.
- Required documentation and user training are provided.

Current evidence position: Not verified. No software build, test execution evidence, production deployment record, training attendance record or signed acceptance form was supplied for this simulation.

6. Outstanding items before a real project could close

- Confirm the approved scope, budget, sponsor and named project team.
- Validate the BRD with actual hospital representatives and obtain formal requirements approval.
- Reconcile the three-month charter target with the approximately 21-week Gantt schedule, including resource capacity and dependencies.
- Complete development and technical testing; retain test evidence and defect-resolution records.
- Validate data migration, data quality, access controls, backup and recovery arrangements.
- Conduct UAT with authorised hospital users and obtain documented acceptance.
- Complete deployment readiness checks, staff training and operational handover.
- Agree ownership and due dates for unresolved defects, risks and deferred features.

7. Lessons learned from the planning exercise

- Requirements should be validated with actual users before they are treated as approved. Assumptions are useful for practice but should be clearly labelled.
- Project dates must be checked against task durations, dependencies and team capacity. A milestone target and a detailed schedule should not contradict each other.
- Progress percentages need a consistent measurement method and reporting cut-off date.
- Risk and issue records serve different purposes: risks describe uncertain future events, while issues describe problems that have already occurred.
- Testing, UAT, deployment, training and handover need evidence; preparing a template does not mean the activity has been completed.
- Patient information requires careful access control, audit trails, secure handling and approved downtime arrangements.

8. Recommendations and future-phase considerations

- Run stakeholder discovery and process mapping with representatives from reception, clinical departments, billing and management.
- Confirm detailed business rules, performance targets, patient portal permissions and data-retention requirements.
- Assess infrastructure and connectivity before committing to online or offline operating arrangements.
- Consider excluded features such as telemedicine, ambulance tracking, advanced pharmacy inventory, full laboratory integration, insurance claims automation and a dedicated mobile application only through a separately assessed change or future phase.

9. Closure decision and approval

A real project manager and authorised sponsor would need to review evidence and formally approve closure.

Approval field	**Record**
Project Manager	Not assigned / not signed
Project Sponsor / Business Owner	Not assigned / not signed
Closure date	Not applicable — simulation only
Decision	Documentation exercise complete; implementation outcome unverified
Outstanding actions	See Section 6